

Where do the health care providers agree?

- AMR has major communication problem
  - Hancock: the public does not understand why resistance is serious when antibiotics usually still appear to work
  - Lester: patients misunderstand resistance and may think themselves are resistant
  - Blondel-Hill: patients often do not understand their diagnosis, screening, or why certain precautions are being taken
  - Liu: the more specialized physicians become, the less they know about the basics and naturally debrief the patient less
    - Liu + Lesley: need to educate nurses more as they spend most time w patients
  - Shared problem identified: AMR information is not being communicated in a way that is accessible or that patients and the public can easily understand
- Patient education needs to improve
  - Hancock: explain the importance of antibiotics and consequences of resistance
  - Lester: provide accessible resources and improve provider-patient communication
  - Blondel-Hill: explain screening, isolation, results
  - Lesley: Many patients have little understanding of antibiotics and AMR
  - Shared solution: health care providers should prioritize patient education as part of their care
- AMR is more than a biological/medical issue
  - Lester: anxiety, stigma, fear, uncertainty, social vulnerability
  - Blondel-Hill: isolation, separation from family, invasive screening, stigma, and reduced interactions with healthcare providers
    - Lesley: infection precautions can lead to fewer visitors and less staff interaction due to fear of infection + additional PPE requirements
  - Shared problem identified: psychosocial burden is important and overlooked
- Patients need a stronger voice in AMR
  - Lester: providers should listen to patient concerns and explain treatment
  - Blondel-Hill: should emphasize empathy
  - Mroz specifically recommends engaging people with lived experience
  - Lesley: patients need to take lead in their own care and be involved

- Liu: v busy healthcare setting and there is a culture where patients do not want to bring issues up
- Shared takeaway: AMR work should be patient-centred

There is a serious gap between how AMR is understood medically and how it is experienced by patients. Insight from the physicians confirm literature findings that the burden of AMR extends far beyond treatment failure. Patients may experience uncertainty, isolation, stigma, repeated screening, communication gaps, and barriers created by prolonged or resource-intensive treatment. These burdens can be amplified by healthcare-system constraints and socioeconomic circumstances. Addressing AMR requires not only better antimicrobials and stewardship, but also accessible education, patient-centred communication, and more emphasis on empathy and psychosocial support.

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## **Hancock**

- Public understanding of AMR is limited because antibiotics still work most of the time (bc docs have backup plans), masking the growing resistance problem
- Emphasize that antibiotics are foundational to modern medicine: surgeries, transplants, neonatal care, and treatment of common infections depend on them
- AMR education should lead with the consequences of losing antibiotics, not just statistics
- Certain populations may face greater AMR risks, inc women (get from pharm w no follow-up), catheterized patients (often wheelchair bound)

## **Lester**

- Patient education and communication should be core part of AMR care
  - Greater emphasis should be placed on training healthcare professionals to communicate to patients how antibiotics work, and why treatment plans are chosen
- Create accessible resources to address common misconceptions: patients may think they are resistant to antibiotics rather than the bacteria thinking all bacteria are bad, misusing antibiotics
- AMR can cause significant psychosocial impacts, including:
  - Anxiety and uncertainty
  - Stigma
  - Fear of transmitting infections to family/friends
  - Feelings of personal responsibility for developing resistant infections

- individuals facing long-term resistant infections (ex. resistant HIV or TB): prolonged psychosocial burdens bc of extended treatment + side effects, often not addressed due to the severity of illness bc constant medical procedures must be done
- + additional vulnerability from pre-existing mental illness, substance use disorders, unstable housing, or lower socioeconomic status
- Healthcare providers should strive to incorporate better social support during their care
- Strengthen antimicrobial stewardship and public awareness.
- Support continued investment in new antimicrobials + improving incentives for pharmaceutical innovation -> don't run out of treatment options

### **Dr Mroz**

- Prioritize patient engagement and lived experiences rather than relying only on clinicians or lit
- Create opportunities for patients to share their experiences and influence the project

### **Dr Blondel-Hill**

- Isolation is a major psychosocial burden for patients with resistant infections, Isolated patients may experience:
  - Fewer/shorter interactions with healthcare providers -> delays in treatment
  - Anxiety, depression, and stigma -> longer recovery time
  - Separation from partners, newborns, and family out of fear to transmit
  - \*future thoughts: Maybe are de-isolation policies bc often patients receive testing once and are isolated -> ensure timely re-assessment to allow out of their isolation if infection-free
- Repeated or invasive screening can make patients feel stigmatized or uncomfortable
- Patients often have confusion about what it is that they have, physicians should be educating
- Some patients may avoid screening after learning they have an AMR risk factor
- inclusive care at patients' bed side, important for physicians to be empathetic towards patients, providers should:
  - Explain screening procedures beforehand, esp for invasive procedures
  - Debrief patients afterward
  - Clearly communicate results
- Reinforce that bacteria are resistant—not the patient.

- Explain geographic/travel-related AMR risks carefully to avoid stigma or perceptions of discrimination - different parts of the world naturally have different rates of AMR prevalence and different bacteria strains

### **Lesley Kooner**

- isolation: infection precautions can lead to fewer visitors and less staff interaction due to fear of infection + additional PPE requirements
- encourage hand hygiene, personal care, appropriate antibiotic use, and awareness of infection symptoms so patients can participate actively in their care
- Many patients have little understanding of antibiotics and AMR
- Need to educate nurses more, stigma exists
- Misconceptions like cloudy urine = UTI, or immediately testing any diarrhea for C. diff can contribute to unnecessary testing and pressure physicians to prescribe antibiotics. Nurses should assess the whole patient first